



The Riverside Practice

EMIS Reception Reference Guide

Standard operating procedures for the reception team

DOCUMENT CONTROL

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WHAT THIS GUIDE COVERS

- System, inboxes, tasks & safety
- Patient details & front-desk requests
- Referrals (standard, BCG, diabetes, retinal)
- Document filing, clinic letters & A&E
- Rejecting documents & prescriptions
- Reports, registration & private letters
- Codes, abbreviations & terms

Living document. This guide is updated as new procedures are learned — the version and date above track each revision (see the revision history overleaf). Everything in EMIS is audited and reversible. Confirm the flagged terms on the index page against what you were shown before relying on them.

Index

Each section is a self-contained workflow. Click an entry (digital copy) or turn to the page number.

1	How the system is laid out	2
1.1	Inbox vs Workflow Manager	2
1.2	Group tasks	2
2	Emergencies & the red button	3
3	Handling tasks & replying	3
4	Editing patient details	3
5	Referrals — the standard flow	4
5.1	Creating the referral letter	4
5.2	Emailing the referral once it's made	4
6	BCG referral (TB vaccine)	4
6.1	Step 1 — Confirm eligibility (check ethnicity)	5
6.2	Step 2 — Create the referral	5
6.3	Step 3 — Send	5
7	Diabetes referral (EDDI)	5
8	Retinal / eye screening	6
9	Document filing & GP Connect	6
9.1	What GP Connect means	6
10	Clinical & clinic letters	6
10.1	If the department isn't stated	7
11	A&E / emergency documents	7
11.1	How to log it	7
12	Codes, abbreviations & terms	7
12.1	Hospital codes	7
12.2	Common abbreviations	8
12.3	Terms worth knowing	8
13	Rejecting documents	8
14	Prescriptions	8
15	Medical reports, registration & private letters	8
15.1	Medical reports & records	9

15.2 Proof of registration confirmation	9
15.3 Private letters (non-NHS work)	9
16 Common front-desk requests	9
16.1 Blood results	10
16.2 Signpost to the online form	10
16.3 Late patients	10
16.4 Pharmacist calls & prescriptions	10

REVISION HISTORY

Version	Date	Change
1.0	18 June 2026	Initial guide compiled from EMIS reception training notes — tasks & inboxes, patient details, referrals (standard, BCG, diabetes, retinal), document filing & GP Connect, clinic letters, A&E coding, rejecting documents and prescriptions.
1.1	18 June 2026	Added <i>Hospital codes & abbreviations</i> reference (HIH = Joerton University Hospital; consolidated RLH and MEH) and the abbreviation d/c = discharged.
1.2	18 June 2026	Added <i>Medical reports, registration & private letters</i> — report/record requests via Accurx (brief vs full, 28 working days, tasked to Iqra), proof of registration (£20), and the private-letter payment process.
1.3	18 June 2026	Added <i>Emergencies & the red button</i> (safety) and <i>Common front-desk requests</i> (blood results, online form signposting, late check-in, pharmacist calls). Added ERS, CD and Mounjaro to the reference section.

Terms to confirm with the practice manager

Spelling has been tidied up, but check these against what you were actually shown:

- **Accurx** — the messaging tool (written “Accurax”). This is the standard NHS GP messaging system that attaches EMIS files and messages professionals — almost certainly what’s meant.
- **BCG** — the TB vaccination referral (written “PCG / PCGM”). The eligibility rules described match BCG, so it is used throughout.
- **EDDI, CEG resource, UmerX**, and the **RP-** form prefix — kept as written; verify the exact names and spellings.
- **Joerton University Hospital (HIH)** — written from speech; this may be **Homerton University Hospital** (East London). Confirm the hospital name and that its code is HIH.

1. How the system is laid out

A few things to keep in your head before anything else.

Inbox vs Workflow Manager

- **My Inbox** — tasks *you* created for yourself (e.g. when you reply to something to follow it up).
- **Workflow Manager** — *everything*: tasks from doctors, tasks assigned to you, and everything else. Opening Workflow shows all tasks broken down by category, and everything listed there is relevant to you.

Group tasks

If a task shows **people icons**, it is a group task — assigned to everyone, not just you.

GOOD TO KNOW

Everything is audited. Any change you make is logged, so it can be reverted if something goes wrong. Don't panic about mistakes — they are recoverable.

LIMIT

You can only have **2 documents open at once** — it won't let you open more. Anything you open is saved in **History**.

2. Emergencies & the red button

There is a **red button in the top right-hand corner** of the desk. It alerts everyone in the practice that there is an emergency, and staff (including some doctors) will respond.

PRESS THE RED BUTTON IF...

- A patient is having a **medical emergency** — for example, they faint.
- You are **in trouble with a patient**, or someone is giving you grief in a way that worries you.

When in doubt, press it — raising the alarm early is always the right call.

3. Handling tasks & replying

Doctors send you tasks through your inbox. Here is how to track and clear them.

1. When a task arrives, add a **sticky note** to keep track of it while you decide what to do.
2. To reply: Reply → Reply to sender
3. Type your message, then press **Send**.
4. Pressing **Send & Complete** marks the task as done automatically.

TO CLOSE A TASK

On My Inbox: Message → Mark done

4. Editing patient details

When a patient sends in updated details (e.g. a new email).

1. Ask the patient for the details and set the message as a **reply** — you can then find all the replies in your **Inbox**.
2. Open User details → Edit

3. Update the record with what they sent you (email, etc.).
4. Save, then **mark the task as done**.

REMINDER

All edits are audited and can be reverted if needed.

5. Referrals — the standard flow

Referrals usually come to you as doctor tasks in your inbox. The shape is the same every time: **create the letter, fill it, email it to the department, mark complete**.

Creating the referral letter

1. Add → Document → Create letter — a form opens showing the patient's details.
2. Click the **magnifying glass** to search forms.
3. Go to Shared folder → Publisher / first folder → Referral forms
4. Find the specific form you need. You can search, but searching brings up several matches.
5. Double-click the form — it opens mostly **pre-filled**. Read through and **verify everything checks out** before editing.

THE "RP" RULE

When you search and several forms come up, pick the one with **RP** in front of it — that is nearly always the correct one.

CRITICAL — OR IT GETS REJECTED

All referrals must go out under Dr Goel's name. If the name is not included, the referral will be rejected.

Emailing the referral once it's made

1. Open **Accurx**: Message → Message professional
2. Click Attach → EMIS file — it shows every document on the patient's record.
3. Select the document you just made — it attaches to the message. The email address is picked up automatically from the document.
4. Send. The referral is now sent.
5. Mark complete: on My Inbox, Message → Mark done

WHERE IT SHOWS UP

A document you create from scratch via Add → Document → Create letter will then appear under **Consultation** alongside anything else you've been working on.

6. BCG referral (TB vaccine)

BCG is arranged by the hospital. The patient is only eligible if a **parent or grandparent** is from a country where tuberculosis is more prevalent.

ELIGIBILITY — CHECK FIRST

Only issue this if a **parent or grandparent is from one of the relevant countries**. The list of countries is in your **email** — check it before proceeding.

Step 1 — Confirm eligibility (check ethnicity)

1. Summary → Investigations → Consultations → Text search
2. Type **ethnicity**.
3. If it lists one of the relevant countries, the patient is eligible.
4. If it is vague, check the **family record**: if the mother or father is from one of those countries, mark them as eligible.

Step 2 — Create the referral

1. In **UmerX**, click **EMIS** — it mirrors onto the actual EMIS too.
2. Add → Document → Create letter
3. Click the magnifying glass → Shared folder → first folder (Publisher) → Referral forms
4. Look for the BCG form — if you can't find it, search and pick the **RP** one.
5. Double-click to open. Most of it is pre-filled — verify, then fill in anything required.

Step 3 — Send

Either send a message or complete the form as required, then email it via Accurx (see the standard flow). Remember: **Dr Goel's name** must be on it.

7. Diabetes referral (EDDI)

When a patient is diagnosed with diabetes there are two strands: an **education programme** (lifestyle and other changes — this is **EDDI**, the standard diabetes referral) and **screening** (see retinal screening, next section). The practice manages blood levels and blood tests; actual eye problems go to the eye clinic.

1. Consultation / Add → Create letter — popup shows the patient's information.
2. Click the magnifying glass → Shared → CEG resource → Referral
3. Look for **diabetes** (you can search — just make sure the form has the **RP** prefix).
4. Fill in the form. **If data isn't available, check Consultations, then Problems.**
5. Save, then email via Accurx: Message professional → EMIS file → select the document you just made — it picks up the email automatically — then send.

HELPFUL

There is already a **new referral document** template that comes pre-filled with the patient's information — saves time.

8. Retinal / eye screening

The screening side of a diabetes diagnosis — diabetic patients need their eyes screened.

1. Add → Add document → magnifying glass (same method as other referrals).
2. Look for **RP retinal** — it should find it.
3. Double-click — the new document is created, mostly filled in already. **Verify everything looks good.**
4. Modify anything you need to. If you need to check details, look in **Problems** as usual.
5. **Save and close.**
6. Send: Message → Message professional → attach EMIS file (the document you just made) — it finds the email automatically — then send.

9. Document filing & GP Connect

Incoming documents land in filing and need sorting.

1. Document management → Awaiting filing
2. Click a document — you'll see options including **GP Connect — update record.**

What GP Connect means

External services (e.g. pharmacies) are given access to our system. **GP Connect means an external provider has entered data onto our records.** After it updates, head to **Consultation** to see the new data. You can tell it's external from the title — it will say something like “pharmacy...”, i.e. not us.

GP CONNECT IS EASY

No changes required from you. For the title, use **date first, then the title.**

10. Clinical & clinic letters

For incoming clinic / clinical documents, the job is to title them clearly and check for any GP actions.

1. Open the document and **keep the type the same.**
2. Change the **date** to the date of the clinic letter (usually stated on it).
3. Add the **hospital and department**. Shortcut: **RLH** = Royal London Hospital. Then paste the department. *If there's no hospital name, just copy the department.*

4. **Read it through** — look for any actions for the GP. If there are none, simply complete the task.
5. In the title, include the date the patient was **last reviewed**, plus anything else useful — **follow-up date**, or a **medication change** if there was one.

WHY THE TITLE MATTERS

Press **F5** for the patient's documents — the list shows each title, so a good title surfaces the useful info at a glance (department, last review, follow-up, medication changes).

If the department isn't stated

Read the letter to find it. A **glaucoma** clinic letter, for example, tells you who reviewed the patient and their profession, and usually names the department. You also need to establish the **date** — infer it from the letter.

11. A&E / emergency documents

Under the GP contract, while patients have access to us during surgery hours, the practice should handle the majority of cases. If a patient goes to A&E for a non-emergency, the practice gets penalised.

WHY THIS MATTERS

Non-emergency A&E attendance during surgery hours = a penalty for the practice. Where it applies, **contact the patient and tell them what they can do instead**. The doctor usually decides whether a case counts.

How to log it

We track these by **coding the document** (setting the document type) — that is what lets us run searches on them later. The A&E document gives you the **date and time** of attendance.

1. Set the **document type** to: Accident and emergency attendance during surgery hours.
2. Add the department. For **Moorfields**, use **MEH**; the document-type department can be **E/R**.
3. Title it the same way as clinic letters (date first, useful info in the title).
4. Copy the document title to a **sticky note** to track it.

12. Codes, abbreviations & terms

Quick reference for the shorthand and terms you'll come across. Add to these as you pick up more.

Hospital codes

Code	Hospital
RLH	Royal London Hospital
MEH	Moorfields Eye Hospital
HIH	Joerton University Hospital

WORTH CONFIRMING

Joerton University Hospital was taken down from speech — it may be **Homerton University Hospital** (East London). Check the name and that the code is **HIH** before relying on it.

Common abbreviations

Short	Meaning
d/c	Discharged
ERS	Electronic Referral System — the electronic referral we send to a specialist
CD	Controlled drug

Terms worth knowing

Term	Meaning
Mounjaro	A weight-loss injection (noted as “Manjaro” — confirm the spelling)

13. Rejecting documents

Some documents simply aren't ours. If a document is **not for our patient**, we don't action it — it's not ours anymore. **Reject it**. You'll be asked for a **reason**, which gets sent back to the hospital.

14. Prescriptions

When a prescription has been prescribed, make sure the medication exists in the system.

1. Check **Medication** and see if the prescribed item is in the database.
2. If it **is** there, you're fine — complete the task.
3. If it **isn't**, email the doctor to bring it to their attention so they can add it. Then complete the task.

READING ANY DOCUMENT — GENERAL CHECKLIST

- Note the **hospital and department**.
- In **Investigations**, look for anything new.

- Check the **Plan** — is it for *us* or for the *patient*? If it's for us, carry out the actions accordingly.

15. Medical reports, registration & private letters

All medical queries and requests are submitted through **Accurx** (the online form).

Medical reports & records

Submitted via the Accurx online form. These can take **up to 28 working days**.

1. When giving this information, make sure the patient **specifies what they want**: a brief summary or their full medical records.
2. **Brief summary** — a short summary of their immunisations, medication and last three consultations (and so on).
3. **Full medical records** — their entire medical record.

WHO HANDLES THESE

All medical report/record requests take **up to 28 working days** and are **tasked to Iqra**.

Proof of registration confirmation

1. Fee: **£20.00**.
2. Ask the patient to **submit a form**.
3. Once that's done, **send them an email** to collect and make payment.

Private letters (non-NHS work)

Private letters are charged from **£20**. Advise the patient that the **doctor reviews the request first**; once approved, we send a message to take payment. Only **after payment is received** does the doctor write the letter and send an SMS to collect.

Process:

1. Patient submits the request. *Online request* → task **Dr Goel**. *Paper form* → put it in the tray with a note requesting authorisation.
2. Once authorised → send an email to make payment.
3. When the patient attends the practice, give them a **receipt** (the invoice book is in the drawer).
4. Put the money in an **envelope with the invoice attached**, and place it in **Dr Goel's tray**.
5. Add a note on **Consultations**: "*Payment received for private letter.*"
6. Once the doctor has written it, he will either **task it back** or **send an email**.

PAYMENT RULE — NO EXCEPTIONS

Take payment **only after the doctor has approved** the request — **never before**. Log everything for audit purposes.

16. Common front-desk requests

A handful of everyday tasks that come up at the desk.

Blood results

A patient asks for their blood results.

1. Diary → Awaiting sample
2. If it isn't there: Investigations → Patient report list → Completed and set the duration to **All**.

Signpost to the online form

For most requests you can direct patients to the **online consultation form** — including registration forms and registration-confirmation requests. (Paid requests such as registration confirmation and private letters still follow the process in *Medical reports, registration & private letters*.)

Late patients

If a patient arrives late, you can **check them in directly** in the booked area.

Pharmacist calls & prescriptions

Pharmacists may call the practice. Keep in mind:

- We **cannot issue medication without a prescription request** — this applies especially to **CD (controlled drug)** items.
- Allow about **3 working days**.